

flora wellness

GUIDE 04 · THE FLORA COLLECTION

The Symptom Decoder

What your body is trying to tell you.

flora wellness collection · guide 04 of 08

\$17

instant download · 34 pages pages

How to Use This Guide

This guide is a practical reference tool for women who want to understand what their symptoms actually mean. It is not a diagnostic manual — it is a decoder. Symptoms are not random. They are your body's way of communicating that something in your hormonal ecosystem needs attention.

HOW TO LOOK UP SYMPTOMS

Use the Symptom Index on pages 25–26 to find your specific symptom alphabetically. Each entry points you to the relevant chapter and page. You can also browse by category: cycle symptoms, skin and hair, energy and weight, or mood and cognition.

SYMPTOMS VS ROOT CAUSES

This guide distinguishes between symptoms (what you feel) and root causes (why it is happening). Heavy periods are a symptom. Estrogen dominance is a root cause. Treating symptoms without addressing root causes provides only temporary relief. This guide always directs you toward the root.

THE CLUSTER EFFECT

Rarely does one hormonal imbalance produce just one symptom. Hormones work in cascades — when one is off, others follow. If you identify with symptoms in multiple chapters, that is common and expected. Look for the overlapping root causes across your symptom cluster.

A NOTE ON MEDICAL ADVICE

This guide is for educational purposes only and does not constitute medical advice. Hormonal health is complex and individual. The information here is intended to help you ask better questions of your healthcare provider, not to replace professional assessment and treatment. Always consult a qualified practitioner before making significant changes to your health protocol.

Table of Contents

Chapter 01	How to Read Your Symptoms	4
	• Symptoms vs Root Causes • The Hormonal Cascade • Why Symptoms Travel in Clusters	
Chapter 02	Cycle Symptoms	6
	• Heavy/Painful Periods • Irregular Cycles • PMS Mood Symptoms • Spotting/Short Cycles • Absent Periods (Amenorrhea) • Cycle Symptom Quick Reference	
Chapter 03	Skin & Hair Symptoms	13
	• Hormonal Acne • Hair Loss/Thinning • Dry Skin/Eczema	
Chapter 04	Energy & Weight Symptoms	17
	• Fatigue • Belly Fat • Unexplained Weight Gain	
Chapter 05	Mood & Cognitive Symptoms	21
	• Anxiety • Brain Fog • Depression	
Chapter 06	Quick Reference Symptom Index	25

How to Read Your Symptoms

Chapter 01

Symptoms are signals, not sentences.

SYMPTOMS VS ROOT CAUSES

A symptom is the message. A hormone is the messenger. When you experience a symptom — bloating, fatigue, mood swings — your body is signalling that something upstream has shifted. Chasing symptoms without identifying the hormonal root cause is like silencing a smoke alarm without finding the fire.

THE HORMONAL CASCADE

Hormones do not operate in isolation. The endocrine system functions as a network of feedback loops. Elevated cortisol suppresses progesterone production. Low progesterone allows estrogen to become dominant. Estrogen dominance impairs thyroid function. Thyroid disruption affects metabolism, mood, and skin. One imbalance cascades into many — which is why symptom clusters are so common.

THE SCIENCE · THE HORMONAL FEEDBACK LOOP

The hypothalamic-pituitary-ovarian (HPO) axis acts as the master regulator. The hypothalamus signals the pituitary, which releases LH and FSH to instruct the ovaries. When estrogen and progesterone feed back to the hypothalamus, they modulate the next cycle of signals. Any disruption — stress, undernutrition, inflammation — distorts this feedback, altering production across all sex hormones.

WHY SYMPTOMS TRAVEL IN CLUSTERS

Because hormones share pathways and precursors, a deficit in one often depletes another. Cortisol and progesterone both derive from pregnenolone — the body prioritises cortisol under stress (the 'pregnenolone steal'), leaving less available for progesterone. This explains why high-stress women frequently experience PMS, poor sleep, and irregular cycles simultaneously.

90%

of symptoms have a hormonal component

4+

average symptoms per cycle

6 months

average delay to hormonal diagnosis

Cycle Symptoms

Chapter 02

Decoding what your period is telling you.

Heavy / Painful Periods

WHAT'S HAPPENING

Estrogen dominance drives excessive endometrial growth during the follicular phase. When the cycle triggers shedding, a thicker endometrium produces more prostaglandins — the inflammatory compounds responsible for cramping. High omega-6 intake and low progesterone both amplify this process, intensifying both blood loss and pain.

THE LIKELY CULPRIT

Excess estrogen · Depleted progesterone · High omega-6 diet · Liver congestion

THE SCIENCE · THE PROSTAGLANDIN CASCADE

Prostaglandins are lipid mediators synthesised from arachidonic acid (an omega-6 fatty acid). Series-2 prostaglandins (PGE2, PGF2α) are pro-inflammatory and drive uterine contraction. Omega-3 fatty acids (EPA/DHA) compete with arachidonic acid, shifting production toward less inflammatory Series-1 and Series-3 prostaglandins — directly reducing cramping severity.

WHAT TO DO

- Increase cruciferous vegetables (broccoli, kale, Brussels sprouts) to support estrogen metabolism via DIM
- Add omega-3 (EPA+DHA 2–3g/day from fish oil) to shift prostaglandin balance
- Magnesium glycinate 300–400mg before bed to reduce uterine cramping
- Consider DIM supplementation (200mg/day) if estrogen dominance is confirmed

SEE ALSO

- [Guide 05: The PMS Elimination Guide](#)
- [Guide 02: The Cycle Syncing Meal Bible](#)

Irregular Cycles

WHAT'S HAPPENING

A regular cycle depends on a well-timed LH surge triggering ovulation. When ovulation is disrupted — by elevated cortisol, thyroid dysfunction, PCOS, or extreme caloric restriction — the cycle length becomes unpredictable. Without reliable ovulation, progesterone output is compromised, compounding hormonal imbalance throughout the cycle.

THE LIKELY CULPRITS

Ovulation disruption · Thyroid dysfunction · PCOS · High cortisol · Undereating or over-exercising

THE SCIENCE · LH SURGE AND OVULATION

The LH (luteinising hormone) surge is the trigger for ovulation, occurring roughly 24–36 hours before egg release. This surge depends on a precise estrogen signal from the maturing follicle. Chronic cortisol elevation suppresses GnRH pulsatility, blunting the LH surge. Thyroid dysfunction impairs estrogen signalling. Both delay or prevent ovulation, extending or shortening cycles unpredictably.

WHAT TO DO

- Track basal body temperature (BBT) daily to confirm whether ovulation is occurring
- Eliminate seed oils and reduce refined carbohydrates to lower systemic inflammation
- Support thyroid with selenium (200mcg) and iodine from whole food sources
- Assess and reduce stress load — consider cycle syncing workouts and sleep prioritisation

SEE ALSO

- [Guide 03: The Hormone Type Guide](#)
- [Guide 01: The 28-Day Hormone Reset](#)

PMS Mood Symptoms

WHAT'S HAPPENING

In the luteal phase, progesterone should rise to its monthly peak. Progesterone metabolises into allopregnanolone, a neurosteroid that binds GABA receptors — the brain's calming system. When progesterone is low, allopregnanolone drops, reducing GABA activity and serotonin availability. The result: irritability, anxiety, tearfulness, and emotional volatility in the 1–2 weeks before menstruation.

THE SCIENCE · PROGESTERONE–GABA CONNECTION

Allopregnanolone (ALLO), a progesterone metabolite, is a positive allosteric modulator of GABAA receptors. GABA is the primary inhibitory neurotransmitter — it reduces neuronal excitability and produces a calming effect. Low progesterone → low ALLO → reduced GABA signalling → increased anxiety, irritability, and sleep disruption. This is also why alcohol (a GABA agonist) temporarily relieves PMS anxiety.

THE LIKELY CULPRIT

Low luteal progesterone · Poor GABA signalling · Low serotonin · High cortisol competing for progesterone precursors

WHAT TO DO

- Seed cycling: flaxseed and pumpkin seeds in follicular phase, sesame and sunflower in luteal
- Vitex agnus-castus (chasteberry) 400mg/day — take for minimum 3 cycles
- Vitamin B6 50–100mg/day in luteal phase to support progesterone and serotonin synthesis
- Magnesium glycinate 300mg at night to enhance GABA receptor sensitivity

SEE ALSO

- [Guide 05: The PMS Elimination Guide](#)
- [Guide 06: The Cortisol Reset](#)

Spotting / Short Cycles

WHAT'S HAPPENING

Spotting between periods and cycles shorter than 24 days typically indicate corpus luteum insufficiency — the structure that forms after ovulation and produces progesterone. When the corpus luteum is underperforming, progesterone levels drop prematurely, causing the uterine lining to begin shedding before the full luteal phase is complete. Short cycles also mean less time in the follicular phase, producing less mature follicles and compounding the problem each cycle.

THE LIKELY CULPRIT

Corpus luteum insufficiency · Inadequate progesterone production · Low LH surge · Chronic stress · Nutritional deficits

THE SCIENCE · CORPUS LUTEUM AND PROGESTERONE

After ovulation, the follicle transforms into the corpus luteum, which produces progesterone for 12–14 days. If insufficient LH stimulation occurs or if the corpus luteum is nutritionally depleted, progesterone output falters. Zinc is essential for LH receptor sensitivity; vitamin C concentrations are highest in the corpus luteum and support its longevity. Deficiencies in either shorten the luteal phase.

WHAT TO DO

- Vitex agnus-castus (chasteberry) to support LH surge and corpus luteum function
- Vitamin B6 and zinc to support progesterone synthesis
- Vitamin C 1000mg/day in luteal phase to support corpus luteum longevity
- Reduce high-intensity exercise in the luteal phase; prioritise stress management

SEE ALSO

- [Guide 05: The PMS Elimination Guide](#)
- [Guide 01: The 28-Day Hormone Reset](#)

Absent Periods (Amenorrhea)

WHAT'S HAPPENING

When periods disappear entirely for three or more months (excluding pregnancy), the most common cause in otherwise healthy reproductive-age women is hypothalamic amenorrhea (HA). The hypothalamus suppresses GnRH pulsatility in response to perceived energy deficit — from undereating, over-exercising, or extreme psychological stress. Without GnRH, the pituitary does not release LH and FSH, and ovulation and menstruation cease entirely.

❑ RED-S: RELATIVE ENERGY DEFICIENCY IN SPORT

- Absent periods in active women are a medical emergency, not a fitness achievement
- RED-S causes bone density loss, stress fractures, and cardiovascular impairment
- The female athlete triad: low energy availability + menstrual disruption + low bone density
- Recovery requires increased caloric intake — not just reduced training volume
- Seek guidance from a sports medicine physician or endocrinologist

WHAT TO DO

- Increase caloric intake to well above your estimated energy expenditure — restoration requires surplus
- Reduce exercise intensity and volume; prioritise rest and walking over high-intensity training
- Assess thyroid function (TSH, free T3, free T4) as hypothyroidism is also a common cause
- Consider working with a registered dietitian experienced in hypothalamic amenorrhea recovery

Cycle Symptom Quick Reference

Use this table to quickly identify the most likely root cause and priority action for each cycle symptom.

SYMPTOM	ROOT CAUSE	KEY NUTRIENT	PRIORITY ACTION
Heavy Periods	Estrogen dominance	Omega-3 / DIM	Cruciferous veg + fish oil daily
Painful Cramps	High prostaglandins	Magnesium glycinate	400mg Mg before bed
Irregular Cycles	Disrupted ovulation	Selenium + iodine	Track BBT; reduce seed oils
PMS Mood	Low progesterone	B6 + Magnesium	Vitex for 3+ cycles
Spotting	Corpus luteum insufficiency	Zinc + Vitamin C	Vitex + reduce luteal exercise
No Period	Hypothalamic suppression	Caloric surplus	Increase food, reduce training

Note: These are common patterns, not absolute rules. Individual variation is significant. Use this table as a starting point for investigation, not a final diagnosis.

Skin & Hair Symptoms

Chapter 03

Your skin is a hormonal report card.

Hormonal Acne

WHAT'S HAPPENING

Hormonal acne results from the intersection of three drivers: androgens (testosterone, DHT), insulin spikes, and inflammatory triggers like dairy. Androgens stimulate sebaceous glands to overproduce sebum; insulin further amplifies androgen sensitivity. The location of acne provides diagnostic clues about which hormones are involved.

ACNE LOCATION MAP

- **Jawline & chin** → *Hormonal — low progesterone / androgen excess*
- **Forehead & nose** → *Digestive and liver congestion*
- **Cheeks** → *Dairy sensitivity / insulin resistance*
- **Back & chest** → *Overall androgen excess / cortisol*

THE SCIENCE · SEBUM PRODUCTION AND ANDROGENS

Testosterone and its more potent metabolite DHT bind androgen receptors in sebaceous glands, upregulating sebum production. Elevated insulin promotes IGF-1, which amplifies androgen receptor activity. Dairy contains IGF-1 precursors and bovine hormones that further stimulate this pathway. Zinc inhibits 5-alpha reductase (the enzyme that converts testosterone to DHT), making it one of the most evidence-backed acne interventions.

WHAT TO DO

- Eliminate all dairy for 30 days as a diagnostic trial — including whey protein
- Reduce refined carbohydrates and sugar to lower insulin and IGF-1 signalling
- Zinc bisglycinate 25–30mg/day — inhibits DHT conversion and supports skin healing
- Spearmint tea 2 cups/day has clinical evidence for reducing androgen levels

SEE ALSO

- [Guide 02: The Cycle Syncing Meal Bible](#)
- [Guide 03: The Hormone Type Guide](#)

Hair Loss / Thinning

WHAT'S HAPPENING

Hair loss is rarely a single-cause issue. The most common drivers are thyroid dysfunction (affecting hair follicle cycling), iron deficiency (ferritin below 70 ng/mL impairs growth), DHT conversion (androgenetic alopecia), and postpartum hormonal crash (telogen effluvium as estrogen drops sharply after birth). All can co-exist.

KEY LAB TARGETS

- **Ferritin**
 > 70 ng/mL for hair *most labs flag 'normal' at 12 — that is not optimal for hair*
- **Free T3**
 Upper quartile *active thyroid hormone; low T3 = hair thinning*
- **Free T4**
 Mid-range minimum *precursor to T3; must test alongside TSH*
- **TSH**
 0.5 – 2.0 mIU/L optimal *mainstream 'normal' range too broad for hair health*

WHAT TO DO

- Test ferritin (not just serum iron) and aim for >70 ng/mL — supplement iron with vitamin C for absorption
- Order a full thyroid panel: TSH + free T3 + free T4 — TSH alone misses most cases
- Biotin 2500–5000mcg/day supports keratin infrastructure — most effective alongside iron and zinc
- Saw palmetto 320mg/day inhibits 5-alpha reductase and reduces DHT-driven follicle miniaturisation

SEE ALSO

- [Guide 04: The Symptom Decoder — Fatigue \(see thyroid testing protocol\)](#)
- [Guide 01: The 28-Day Hormone Reset](#)

Dry Skin / Eczema

WHAT'S HAPPENING

Chronically dry skin and eczema-type flares are frequently hormonal in origin. Thyroid hormones (particularly T3) directly regulate skin cell turnover, sebaceous gland activity, and the synthesis of hyaluronic acid — the skin's internal moisture reservoir. Subclinical hypothyroidism produces noticeably dry, rough skin long before other symptoms appear. Essential fatty acid deficiency further compromises the skin barrier.

THE LIKELY CULPRIT

Subclinical hypothyroidism · Essential fatty acid deficiency · Low estrogen · Gut dysbiosis · Liver overload

THE SCIENCE · THYROID AND SKIN BARRIER FUNCTION

T3 receptors are expressed in keratinocytes (skin cells) and sebaceous glands. When T3 is low, keratinocyte proliferation slows, sebum production drops, and the stratum corneum loses structural integrity — allowing transepidermal water loss. Simultaneously, hyaluronan synthase expression decreases, reducing the skin's capacity to bind water. These changes appear as dryness, flaking, and impaired wound healing.

WHAT TO DO

- Omega-3 (EPA+DHA 2g/day) to restore skin barrier lipids and reduce inflammatory drive
- Evening primrose oil 1–2g/day (GLA content) specifically targets eczema and barrier function
- Full thyroid panel (TSH + free T3 + free T4) — subclinical hypothyroidism is frequently missed
- Reduce ultra-processed foods and assess for food sensitivities (gluten, dairy are common triggers)

SEE ALSO

- [Guide 03: The Hormone Type Guide](#)
- [Guide 02: The Cycle Syncing Meal Bible](#)

Energy & Weight Symptoms

Chapter 04

Fatigue and weight changes are never random.

SYMPTOM 09

Fatigue

WHAT'S HAPPENING

Persistent fatigue in women is rarely about sleep alone. The four most common hormonal drivers are cortisol dysregulation (flat or inverted diurnal rhythm), thyroid dysfunction (even when TSH is 'normal'), iron deficiency anemia or low ferritin, and B12 depletion. These frequently coexist and each amplifies the others.

73%

of women with fatigue have thyroid issues

1 in 5

women are iron deficient

B12

deficiency mimics depression

THE LIKELY CULPRITS

Cortisol dysregulation · Subclinical hypothyroidism · Low ferritin · B12 deficiency · Blood sugar instability

WHAT TO DO

- Order a full thyroid panel (TSH + free T3 + free T4) — not TSH alone — and ferritin, B12
- Four-point salivary cortisol test to map your diurnal cortisol rhythm and identify dysregulation
- B12 (methylcobalamin 1000mcg sublingual), especially if plant-based or on metformin
- Protein at every meal to stabilise blood sugar — start with 30g at breakfast

SEE ALSO

- [Guide 06: The Cortisol Reset](#)
- [Guide 01: The 28-Day Hormone Reset](#)

SYMPTOM 10

Belly Fat

WHAT'S HAPPENING

Visceral fat accumulation around the abdomen is driven primarily by two hormonal forces: elevated cortisol and insulin resistance. Visceral adipose tissue has a high density of cortisol receptors, making it preferentially targeted for fat storage when cortisol is chronically elevated. Simultaneously, insulin resistance creates a state of 'fat lock' — the body cannot access stored fat for energy and continues depositing more.

THE SCIENCE · CORTISOL-ADIPOSE TISSUE CONNECTION

Visceral adipocytes express 11 β -HSD1, an enzyme that locally converts inactive cortisone to active cortisol. This creates a self-amplifying loop: stress → systemic cortisol → local cortisol amplification in visceral fat → increased fat deposition → more cortisol receptors expressed. Cortisol also promotes insulin resistance by impairing GLUT4 glucose transporter activity, creating a compounding metabolic problem.

THE LIKELY CULPRIT

Chronically elevated cortisol · Insulin resistance · Poor sleep · Low progesterone · Excess alcohol

WHAT TO DO

- Protein at every meal (aim for 30g minimum) to reduce cortisol-driven muscle breakdown
- Resistance training 3x/week — builds insulin-sensitive muscle tissue and reduces visceral fat
- Ashwagandha 600mg/day (KSM-66 extract) has clinical evidence for reducing cortisol and belly fat
- Reduce or eliminate alcohol — disrupts cortisol rhythm and drives visceral fat storage

SEE ALSO

- [Guide 06: The Cortisol Reset](#)
- [Guide 07: The Cycle Fitness Guide](#)

SYMPTOM 11

Unexplained Weight Gain

WHAT'S HAPPENING

Weight gain despite no change in diet or activity is almost always hormonal. The three most common causes are subclinical hypothyroidism (slowing metabolic rate), estrogen dominance (promoting water retention and fat storage particularly around hips and thighs), and insulin resistance (preventing cellular glucose uptake and promoting fat storage). Often all three coexist and reinforce each other.

THE LIKELY CULPRIT

Subclinical hypothyroidism · Estrogen dominance · Insulin resistance · Cortisol dysregulation

THE SCIENCE · THYROID AND METABOLIC RATE

Thyroid hormones (T3/T4) regulate the basal metabolic rate by controlling mitochondrial activity and thermogenesis. Even mild reductions in free T3 — which standard TSH testing often misses — can reduce BMR by 10–15%. This translates directly to weight gain without any change in caloric intake. Additionally, low T3 reduces SHBG (sex hormone binding globulin), allowing more free estrogen to circulate and worsen estrogen dominance.

WHAT TO DO

- Eliminate ultra-processed foods and artificial sweeteners — both impair insulin signalling
- Get TSH + free T3 + free T4 tested — subclinical hypothyroidism is the most missed cause
- Magnesium glycinate 300–400mg/day improves insulin sensitivity at the cellular level
- Cruciferous vegetables daily to support estrogen clearance via the liver's glucuronidation pathway

SEE ALSO

- [Guide 03: The Hormone Type Guide](#)
- [Guide 02: The Cycle Syncing Meal Bible](#)

Mood & Cognitive Symptoms

Chapter 05

Hormones write your emotional script.

Anxiety

WHAT'S HAPPENING

Hormonal anxiety has two primary drivers: low progesterone reducing GABA activity, and chronically elevated cortisol amplifying the amygdala's threat response. Progesterone metabolises into allopregnanolone, which calms the nervous system via GABA receptors. When progesterone is low (especially in the luteal phase or during perimenopause), this calming mechanism is impaired, leaving the nervous system in a heightened state.

THE SCIENCE · PROGESTERONE-GABA CONVERSION

Progesterone is enzymatically converted to allopregnanolone (ALLO) in the brain and periphery. ALLO is a potent positive allosteric modulator of GABAA receptors — the same receptors targeted by benzodiazepines. When progesterone drops in the late luteal phase or during perimenopause, ALLO levels fall, reducing GABA receptor sensitivity. Cortisol simultaneously increases CRH (corticotropin-releasing hormone), directly stimulating the amygdala and escalating anxiety, especially around perceived threats.

THE LIKELY CULPRIT

Low luteal progesterone · Elevated cortisol · Low GABA signalling · Blood sugar instability · Excess caffeine

WHAT TO DO

- Magnesium glycinate 300mg evening — most bioavailable form for GABA receptor function
- L-theanine 200mg — crosses the blood-brain barrier and directly supports GABA/alpha wave activity
- Reduce or eliminate caffeine, especially after midday — caffeine blocks adenosine and elevates cortisol
- Vitex agnus-castus if anxiety is specifically luteal — supports progesterone production
- Therapy (CBT or somatic approaches) alongside nutritional support for comprehensive outcomes

SEE ALSO

- [Guide 06: The Cortisol Reset](#)
- [Guide 05: The PMS Elimination Guide](#)

SYMPTOM 13

Brain Fog

WHAT'S HAPPENING

Brain fog — the inability to think clearly, poor memory, difficulty concentrating — has three common hormonal roots: thyroid dysfunction slows overall neural metabolism; estrogen fluctuation (particularly the drop before menstruation or during perimenopause) impairs memory consolidation and verbal recall; and blood sugar instability deprives the brain of its primary fuel, impairing executive function and focus.

THE LIKELY CULPRIT

Subclinical hypothyroidism · Estrogen fluctuation · Blood sugar instability · Inflammation · B12 deficiency

THE SCIENCE · ESTROGEN AND COGNITIVE FUNCTION

Estrogen receptors (ER α and ER β) are highly expressed in the hippocampus and prefrontal cortex — the brain regions most critical for memory and executive function. Estrogen promotes synaptic plasticity, supports acetylcholine synthesis (the learning neurotransmitter), and increases cerebral blood flow. When estrogen fluctuates dramatically — as it does before menstruation or in perimenopause — these cognitive functions are transiently impaired.

WHAT TO DO

- Protein breakfast within 60 minutes of waking to anchor blood sugar and fuel morning focus
- Omega-3 (EPA+DHA 2g/day) — DHA is the primary structural fat in brain tissue
- Full thyroid panel — free T3 is the active hormone for brain metabolism; TSH alone is insufficient
- Trial gluten elimination for 30 days — gluten sensitivity correlates with neurological symptoms in susceptible individuals

SEE ALSO

- [Guide 01: The 28-Day Hormone Reset](#)
- [Guide 02: The Cycle Syncing Meal Bible](#)

Depression

WHAT'S HAPPENING

Hormonal depression is driven by two primary mechanisms: the drop in allopregnanolone (a progesterone metabolite with antidepressant properties) during the luteal phase or postpartum period, and depletion of B vitamins required for serotonin synthesis. Low B6 impairs the conversion of tryptophan to serotonin; low folate and B12 reduce methylation, impairing neurotransmitter production across multiple pathways.

THE SCIENCE · ALLOPREGNANOLONE AND MOOD

Allopregnanolone (ALLO) is now recognised as a key antidepressant neurosteroid. The FDA approved brexanolone (synthetic ALLO) in 2019 for postpartum depression — the first treatment targeting this mechanism. ALLO acts at GABAA receptors and modulates neuroplasticity pathways. Its sharp decline postpartum (as progesterone falls) or during the late luteal phase explains the cyclical nature of hormonal depression. ALLO also protects against neuroinflammation — a key factor in chronic depression.

THE LIKELY CULPRIT

Low allopregnanolone · B vitamin depletion · Omega-3 deficiency · Low thyroid · Light deficiency · Inflammation

WHAT TO DO

- B-complex (methylated forms: methylfolate, methylcobalamin) to support serotonin and dopamine synthesis
- Saffron extract 30mg/day — multiple RCTs show comparable efficacy to SSRIs for mild-moderate depression
- Omega-3 (EPA 1.5–2g/day specifically) — EPA is the anti-inflammatory fraction with the strongest mood evidence
- Light therapy 10,000 lux morning — effective for seasonal and non-seasonal depression, regulates circadian rhythm
- Seek medical support — depression is serious and nutritional strategies work best alongside professional care

SEE ALSO

- [Guide 05: The PMS Elimination Guide](#)
- [Guide 06: The Cortisol Reset](#)

Symptom Index A – M

SYMPTOM	HORMONE / CAUSE	LOCATION
Absent Periods	Hypothalamic suppression / low FSH/LH	Ch. 2 · p.11
Acne (hormonal)	Androgens + insulin + dairy	Ch. 3 · p.14
Anxiety	Low progesterone / elevated cortisol	Ch. 5 · p.22
Belly Fat	Cortisol / insulin resistance	Ch. 4 · p.19
Brain Fog	Thyroid / estrogen / blood sugar	Ch. 5 · p.23
Cramps (severe)	High prostaglandins / estrogen dominance	Ch. 2 · p.7
Depression	Low allopregnanolone / B vitamins	Ch. 5 · p.24
Dry Skin	Low thyroid / EFA deficiency	Ch. 3 · p.16
Eczema	Low thyroid / gut dysbiosis	Ch. 3 · p.16
Fatigue	Thyroid / cortisol / iron / B12	Ch. 4 · p.18
Hair Loss	Thyroid / iron / DHT / postpartum	Ch. 3 · p.15
Hair Thinning	Low ferritin / thyroid dysfunction	Ch. 3 · p.15
Heavy Periods	Estrogen dominance	Ch. 2 · p.7

Symptom Index N – Z

SYMPTOM	HORMONE / CAUSE	LOCATION
Irregular Cycles	Ovulation disruption / PCOS / thyroid	Ch. 2 · p.8
Irritability (PMS)	Low progesterone / low GABA	Ch. 2 · p.9
Low Mood	Low allopregnanolone / omega-3	Ch. 5 · p.24
Memory Issues	Estrogen fluctuation / low T3	Ch. 5 · p.23
Mood Swings	Estrogen / progesterone imbalance	Ch. 2 · p.9
Night Sweats	Declining estrogen / cortisol spikes	Ch. 4 · p.18
PMS	Low luteal progesterone	Ch. 2 · p.9
Short Cycles	Corpus luteum insufficiency	Ch. 2 · p.10
Spotting	Low progesterone / endometrial instability	Ch. 2 · p.10
Thyroid Symptoms	TSH/T3/T4 dysregulation	See all chapters
Weight Gain	Thyroid / estrogen / insulin resistance	Ch. 4 · p.20
Weight Loss Resistance	Cortisol / insulin / thyroid	Ch. 4 · p.19

This index covers the primary symptoms discussed in this guide. For a deeper investigation of any root cause, refer to the relevant chapter. Symptoms often overlap — check multiple entries if you identify with several.

flora wellness

The Flora Collection

01 · The 28-Day Hormone Reset

02 · The Cycle Syncing Meal Bible

03 · The Hormone Type Guide

04 · The Symptom Decoder

05 · The PMS Elimination Guide

06 · The Cortisol Reset

07 · The Cycle Fitness Guide

08 · The 90-Day Rebalancing Roadmap

For educational purposes only. Not medical advice.